5 Parenting support for refugee and vulnerable communities in Lebanon and Jordan during COVID-19 and a humanitarian crisis

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Introduction

Lebanon and Jordan have been hosting refugees and internally displaced persons for decades. The 2011 Syrian conflict caused a large influx of refugees to flee to both of these countries, but even before that, Lebanon and Jordan hosted other refugees and dealt with internal political and socio-economic conflict (Karasapan, 2022). The COVID-19 pandemic added an additional burden on government systems in both countries, particularly in the areas of education, social welfare, and health, which were already struggling to meet the needs of children in their countries (Al-Mulki et al., 2022; Alshoubaki & Harris, 2021). At the end of 2019, a political crisis in Lebanon further deteriorated the situation in the country, creating even more challenges to providing early childhood development services (Al-Mulki et al., 2022).

The Arab Resource Collective (ARC) is a non-governmental organization founded in 1989 to address a lack of health and development support in the region. The organization had a focus on early childhood development (ECD) since the beginning and in 2014 established the Arab Network for Early Childhood (ANECD), which specifically supports ECD in Arab countries. With COVID-19 adding to existing burdens of conflict and political crises in both Jordan and Lebanon, ARC adapted its existing parenting program – the Health, Early Learning, and Protection Parenting Program (HEPPP) – to better address the needs of parents and caregivers of young children from birth to eight years. The HEPPP aims to strengthen parents' parenting knowledge, attitudes, and practices so they can provide the best support to their young children. It addresses mothers and fathers together as a unit and has been used with Syrian and Palestinian refugees and host communities in Lebanon, Jordan, and Egypt. The HEPPP was initially developed by multi-sectoral Arab experts in 2012 to fill a gap in supporting parents of young children in Arab countries. It is based on evidence that children need holistic support for health, education, and

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protection (Lancet, 2016; WHO, UNICEF, World Bank, 2018). A strong body of evidence shows that caregivers are children's first, most important, and constant teachers and that in order for young children to thrive, caregivers need support, too (Britto et al., 2017).

With the COVID-19 pandemic, the ARC made several modifications to the existing HEPPP model that had been delivered exclusively using a face-to-face modality. When parents could not gather in person as they did before, ARC wanted to ensure the continuation of services for parents/caregivers of young children. The HEPPP model added some important elements such as psychosocial support for children and parents/caregivers. The model was adapted to be delivered fully online, but was also delivered in-person using two different approaches (both described below).

ARC and its implementing and research partners – Plan International Jordan (PI-Jo), the American University of Beirut (AUB), and the Modern University for Business and Science (MUBS) – conducted research between 2019 and 2021 to compare two versions of the face-to-face and one version of the online adaptations. Since the COVID-19 pandemic has eased, ARC innovated further to include a hybrid modality that included both face-to-face and online support, which will be evaluated in 2023.

This chapter outlines the HEPPP program's content and modifications made during COVID-19. It highlights key decisions that were made while developing these changes and shares the results of research that compared the face-to-face and online modalities. The chapter shares challenges of doing research in unstable conditions such as multi-humanitarian crises, comments on advantages and disadvantages of each program modality tested, and reflects upon the application of these approaches in other crisis contexts. While the evaluation identified challenges with program delivery, there were positive changes in the knowledge, behaviors, and practice of participating parents, which could have implications for their children's well-being around the world and in various contexts.

Context of refugee and vulnerable children and families in Lebanon and Jordan

Lebanon

Lebanon has the world's highest proportion of refugees per population (one-third of the population). Approximately 1.5 million Syrian refugees are in Lebanon (UNHCR, n.d.), and millions of Palestinians have been in Lebanon for decades. Lebanon was already strained by the influx of refugees within its borders when the COVID-19 pandemic hit. The pandemic further posed challenges to young children's learning and development. While the pandemic was raging in Lebanon, the situation further deteriorated dramatically with a political and economic crisis.

Since the end of 2019, Lebanon has been suffering from severe economic and financial crisis, which has affected a large percentage of the Lebanese population, including refugees residing in Lebanon (World Bank, 2022). A United Nations report estimated that the proportion of the population living in multidimensional poverty nearly doubled between 2019 and 2021, from 42% to 82% (UNESCWA, 2021). Young Lebanese and refugee children and their families have been directly affected by daily power outages, shortages in medical supplies, vaccines, fuel, and other essentials, inadequate safe drinking water, and high food prices (Bonet, 2022). There are also insufficient staff to support children (such as medical professionals and ECD professionals who have been leaving the country or the sector) (Mroue, 2021; UNICEF Lebanon, 2022; Bonet, 2022; Dadouch & Durghan, 2021). Many children have not gone to school (even when schools were open) because of the high cost of transportation, books, and materials. A 2018 report estimated that less than half of the more than 600,000 school-aged children in Lebanon were enrolled in school (Human Rights Watch, 2018). Children are exposed to violence in the streets, and there has been an increase in domestic violence in homes (ANECD, 2022).

A 2021 study by ANECD and their members in six Arab countries on the situation of young children's families after one year of the COVID-19 pandemic found many difficulties (ANECD, 2022). The survey interviewed 486 Lebanese, Syrian, and Palestinian families living in Lebanon (ANECD, 2022). Adults from these families expressed the need for financial support, food rations, educational materials, and psychosocial and mental health support. For their children, they requested – in order of priority – food and nutrition support, learning opportunities, psychosocial and mental health support, and physical health support. Of the participating families, 38% had not completed routine vaccines for their children. The study found that emotional and behavioral problems were increasing among children and psychosocial well-being was worsening: 45% of children surveyed exhibited more anger, 31% cried more, and 22% had more difficulty sleeping than before. Parents and caregivers were also experiencing challenges of the multi-crisis situation; 75% of parents stated feeling more mood changes and becoming more nervous than before COVID-19. About a third of families surveyed did not feel they could control themselves and 54% of these adults stated they resorted to violence or shouting at their children and in the home. Lastly, 38% of surveyed families complained of not being able to maintain normal activities or being unable to create appropriate activities for their children (ANECD, 2022).

With the political crisis in Lebanon at its worst, government bodies working at a slow pace and government employees going on strike, Lebanon has become even more vulnerable. The ANECD report showed that most respondents did not receive any aid or support, especially from the government (only 15% of Syrians, 6% of Lebanese, and 3% of Palestinians received aid). The support received was mostly financial and food packages, and ignored all their other needs (ANECD, 2022).

As in many countries, the responsibility for holistic early childhood development rests across multiple government ministries, including education, health and social services. Each ministry focuses on a particular element of ECD. For example, the Ministry of Education in their Education plan for 2021–2025 aims to increase access of one year of pre-primary education from 50% in 2019–2020 to 90% by 2025 (Ministry of Education Lebanon, 2021). The Ministry of Health's strategic plan includes indicators for reducing maternal and under five child mortality and immunization and health education support for school-based health programs (Ministry of Health Lebanon, 2007). While specific policies exist for the different ministries, there is no comprehensive national ECD strategy. Lebanon country profile review conducted for the Nurturing Care Countdown to 2020 found a lot of missing information in terms of policies and data for young children (UNICEF, n.d.). A positive step is that an ECD National Committee will be established in April 2023 to develop a national ECD strategy. ARC and its partners have been working closely with various government departments to support this process and ensure parenting is included.

Jordan

Jordan has been facing a multi-crisis situation for many years. Jordan hosts over 758,000 refugees and asylum seekers, and 12.5% of these are under five years old. Of these, approximately 88.5% are Syrian, 8.8% are Iraqi, 1.7% are Yemeni, and the rest from Somalia, Sudan, and other countries (WHO, 2023). Jordan also hosts approximately 2 million Palestinian refugees who are registered with the United Nations Relief and Works Agency for Palestine Refugees in the Near East (UNRWA) (UNRWA, n.d.) Syrian refugees and over 90,000 refugees of other nationalities live in two main camps: Azraq and Za'atari. Azraq Camp, which is the location of the Arab Resource Collective's early childhood development work, is home to 37,129 Syrian refugees, and nearly 21% are under five-years-old (UN Jordan, 2022; UNHCR, 2019). A 2022 study estimates that 66% of refugees live below the international poverty line of $5.5 USD per person per day (UNHCR/World Bank, 2023); but children experience higher rates of poverty than adults (UNICEF, 2017). The United Nations High Commission for Refugees (UNHCR) reported that the economic downturn prompted by the COVID-19 pandemic has pushed 49,000 Syrian refugees already in Jordan into an ever more desperate situation and has increased their humanitarian needs (UNHCR, 2020).

Overall, most young children in Jordan have access to vaccines and health services (UNICEF, 2022). However, Syrian refugees have worse health and nutrition outcomes (UNHCR, UNICEF, WFP, 2016). Child caring practices are insufficient. According to a survey on early childhood by the Queen Rania Foundation in 2015, more than 40% of mothers reported that they or other family members never read or look at picture books with their children under the age of five. Further, the survey found that 65% of households had no children's books. Approximately one in three mothers of children aged four to five reported that they or other family members never took their children to a play area or a park, and 40% never played sports or active games or exercises with them (Hatamleh et al, 2015).

ANECD conducted research on the situation of young children and families after one year of the COVID-19 pandemic in Jordan with the National Council for Family Affairs (NCFA) (ANECD, 2022). The study included 1,172 Jordanian, Syrian, and Palestinian families living in Jordan. Of the participating families, 95% of caregivers expressed needing support to deal with the child. Large numbers expressed the need for financial support (50% of Jordanians and 90% of Syrian refugees), educational material and toys (56% of Jordanians and 50% of Syrian refugees), and psychological support (41% of Jordanians and 30% of Syrian refugees). The report shows that 50% of children surveyed were exhibiting more anger, 29% crying more, and 36% having more difficulty sleeping than before (ANECD, 2022).

In Jordan, the responsibility for early childhood development is spread among the Ministry of Education, Ministry of Social Development, Ministry of Awqaf and Islamic Affairs, and the Ministry of Health. In 2001, Queen Rania founded the National Council for Family Affairs (NCFA). It is an a civil society umbrella organization that supports, coordinates and facilitates the work of its partners and relevant government institutions which are involved in and important for family affairs (NCFA, n.d.). A number of policies exist in Jordan which include some supports for young children such as the National Education Strategic Plan (2018–2022), the Jordanian ECD Strategy and the national plan of action (developed by the NCFA) and a National Nutrition Strategy (Ministry of Education Jordan, n.d.; NCFA, n.d.; WHO, 2022). Unfortunately, these policies do not provide holistic support to all children from birth to five years. For example, the Education plan covers KG1 and KG2 (pre-primary education for four and five year olds), but does not include health and nutrition support. The health and nutrition policy includes nutrition and health support for children under three years, but do not include early stimulation and learning for this age group. A Jordan country profile review conducted for the Nurturing Care Countdown to 2020 found a lot of missing information in terms of policies and data for young children (UNICEF, n.d.).

Key stakeholders in the country, such as UNICEF, Plan International and others, are working closely with all ministries to ensure children's holistic development is a part of all existing policies. UNICEF and other partners are supporting the various government ministries to increase holistic ECD services through an increase in support for parenting programs, nurseries for children under five years and kindergarten access for children five years and older (UNICEF, 2019).

Importance of parents and caregivers

Early life experiences set the foundation for brain development. Interactions between children and their parents and caregivers play a critical role in helping them reach their developmental potential (Gee, 2016; Britto et al., 2015). According to the American Academy of Pediatrics, safe, stable, nurturing relationships (SSNRs) are biological necessities for all children (Garner and

Yogman 2021). These relationships are "potent antidotes for childhood adversity" as they help children regulate and turn off the body's stress machinery in a timely manner (Garner and Yogman 2021). Parents and caregivers have the most powerful impact on children, especially throughout their early childhood years. Parents and caregivers may need support in carrying out their responsibilities and creating optimal environments for positive child development if they are hampered by risk factors such as poverty.

Studies show that parenting styles and practices significantly affect and play a vital role in early childhood development (Smith et al., 2005; Parke, 2002; Gershoff, 2002; Eisenberg et al., 2001; Gee, 2016; Britto et al., 2015; Dardas & Ahmad, 2014; Shah et al., 2016). Regardless of culture, studies show that responsive parenting and caregiving, which refers to the ability of the parent and/or caregiver to "notice, understand and respond to their child's signals in a timely and appropriate manner" (Nurturing Care Framework, n.d.), improves children's cognitive and linguistic development (Shah et al., 2016). When parents themselves are struggling, especially in conditions of crisis and stress, this can affect their parenting abilities, with implications for their children. Indeed, parental stress has been found to have a deleterious effect on children's wellness and behaviors (e.g., newborn temperament, infant negative affectivity, and behavioral issues at the age of three to nine years) (Franco et al., 2010; Chang et al., 2004; Neece et al., 2012).

Many organizations have been implementing programs targeting parents and caregivers. These parenting programs aim to increase caregivers' knowledge and improve their attitudes, behaviors, and practices so they can best support young children's positive development. Many programs also help parents manage their own stress. These programs range from individual home or clinic visits to community-based group sessions with or without home visits (Britto et al., 2015; Aboud & Yousafzai, 2015; Yousafzai & Aboud, 2014). To date, most programs have focused on mothers and few have involved fathers (Britto et al., 2015). Successful implementation strategies included a planned, structured curriculum, demonstrations with materials, the chance to practice new skills with pertinent feedback, and problem-solving dialogues with other parents (Yousafzai & Aboud, 2014). Parenting programs that encourage positive parenting behaviors, skills, and disciplines while enhancing parents' and caregivers' well-being and minimizing parenting stress are needed worldwide, particularly in impoverished and vulnerable communities (Eruyar et al., 2018).

Evidence to date on parent and caregiver support programs has shown an improvement in children's emotional and behavioral outcomes as well as parents' psychological well-being.[24] Some parenting programs in low- and middle-income countries have integrated child psychological and nutrition stimulation to address co-occurring risks and showed a significant positive effect on children and/or parenting outcomes (Britto et al., 2015; Grantham-McGregor et al., 2013). Some studies have also shown that parenting programs for refugee and migrant parents have improved parenting efficacy, positive parenting behaviors, parent-child relationships, parenting stress, parental practices, and families' well-being (Hamari et al., 2021; Lee and Kim, 2022). In 2015, UNICEF conducted a systematic review of parenting programs in low- and middle-income countries (Britto et al., 2015). The review found that dose is important. Programs needed to be at least 12 months to have an impact on children's outcomes and programs lasting more than two years produced more consistent impact. Modality also played a role in impact. Child cognitive outcomes, for example, improved significantly in group-setting programs with a psychosocial stimulation portion (i.e., active engagement between the caregiver and the child), whether the programs were home-based or center-based. The review also highlighted the importance of having well-trained and supervised paraprofessionals deliver programs and noted that including fathers is a promising and underutilized strategy (Britto et al., 2015).

Even before COVID-19 affected the world, some organizations tested web-based parenting programs. A 2013 meta-analysis of web-based parenting programs found that online interventions can be suitable for providing information to parents and training them on parenting skills. The review found that guided and self-guided online interventions can contribute to moderately significant effects on parent and child outcomes (Nieuwboer et al., 2013). A 2020 meta-analysis found that online parenting programs provide positive support to parents who cannot attend in-person sessions (face-to-face). Improvements were related to positive parenting and parents' encouragement as well as improved parent confidence, child behavior, and parenting satisfaction. Other significant effects were a reduction in harmful parent-child interactions, problematic child behaviors, negative discipline strategies, parent stress, anger, and depression, and child anxiety (Spencer et al., 2020). Furthermore, there were no statistically significant differences in the outcomes between online parenting programs with and without clinical support (i.e., programs where participants had more access to content experts, therapists, or content specialists) or between programs provided to targeted versus general populations (Spencer et al., 2020).

While online parenting programs have increased dramatically during the pandemic, research on them is still limited, especially in the Arab World and multi-crisis contexts. A few studies show some evidence on parenting programs in the region. In 2017–18, the Arab Resource Collective (ARC) in Lebanon, in coordination with Plan International in Jordan, conducted a pilot cohort study of an intervention promoting positive parenting in Syrian refugees in Lebanon and Jordan. The intervention was delivered face-to-face by master trainers. The study revealed that the initial version of the parenting intervention improved various parenting practices and boosted parents' wellbeing (Lakkis et al., 2020). In 2017–2019, before the COVID-19 pandemic, War Child Holland undertook a randomized controlled trial (RCT) of a pilot intervention with Syrian refugees and host country families in North Lebanon. By the end of this study, the intervention group experienced improved parental mental well-being and stress management, increased parental warmth and responsiveness, and decreased harsh parenting (Miller et al., 2020). Additionally, parents who received the program reported increased child psychosocial well-being (Miller et al., 2020). There was no significant change in any variable in the control families (Miller et al., 2020). Another RCT was conducted in Greater Beirut, Lebanon to evaluate a preventive parenting program designed by local experts for the local cultural context (Dirani et al., 2021). However, it did not produce significant effects despite the positive feedback from mothers who attended the sessions (Dirani et al., 2021). Given the limited evidence in the Arab region the findings in this chapter fill a critical gap in the evidence base.

Health, Education, Protection, Parenting Program (HEPPP)

The Health, Early Learning, Protection, Parenting Program (HEPPP) was one of the first parenting models in the Arabic-speaking world that integrated all elements of children's holistic needs. It was first developed in 2012 by a group of experts including pediatricians and experts in mental health, early learning, and parenting in the Middle East. The program provides a framework of concepts, knowledge, and exercises that help develop parents' knowledge, behaviors, and practices to support their children's positive development outcomes. A core element of the HEPPP model, which is different from many other parent support programs, is that it is implemented with couples – both mothers and fathers– rather than just mothers, which is the case with most parenting programs. This "added value" reaffirms the father's role as an essential partner with the mother in their children's development and well-being.

The HEPPP model consists of various interactive parenting sessions that support parents from the period of pregnancy until their child is 8 years old. It aligns with ARC's holistic approach to ECD (see Figure 5.1) and includes the following broad topics:

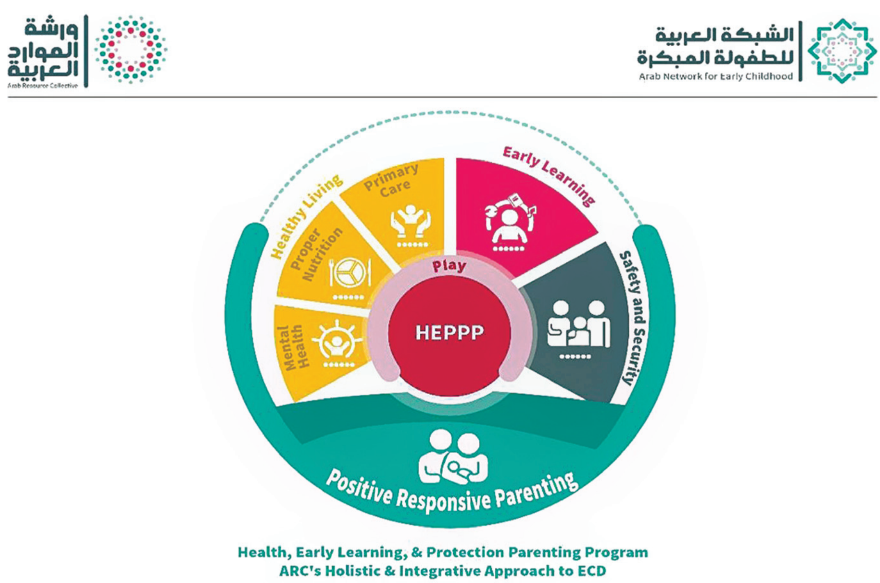

Figure 5.1 Health, Early Learning and Protection Parenting Program Model

1 Introduction to early childhood development (including brain development, physical and cognitive development, and emotional and social development)
2 Communication between caregivers and child and between children and their siblings and peers
3 Managing behavioral problems in early childhood (i.e., self-control and use of a non-violent approach with children)
4 Play and early childhood (games that parents and children can play at home to support their development)
5 Safety and prevention of accidents
6 Personal hygiene and protection from diseases
7 Balanced nutrition for children
8 Reinforcing justice and equality between children
9 Multiple intelligences and learning
10 Kindergarten and school preparedness

The curriculum initially consisted of a total of 21 sessions, including 16 face-to-face sessions for parents to learn about child development and how to best understand and attend to their children's needs, plus five sessions focused on helping them improve their own mental health and psychosocial well-being.

After eight years of experience implementing the original curriculum in cities, rural areas, and refugee camps in Lebanon, Jordan, and Egypt, ARC learned a lot and began to make modifications. Some sessions, such as ones focused on breastfeeding, discouraged fathers from participating because they felt it was not relevant to them. In response, ARC adapted the content and order of sessions to ensure it appealed to both mothers and fathers.

Digital adaptation of HEPPP during COVID-19

In 2020 and based on recommendations and findings from eight years of piloting and implementation in Lebanon, Jordan, and Egypt, the ARC team, along with more than 20 experts from the Arab world, updated the curriculum to better reflect the Arab context and culture, and the updated science of ECD. The updating and expansion of the HEPPP are still ongoing. To address cost and parent flexibility, ARC had already been thinking about digitizing the full HEPPP curriculum and making it fully open-sourced. The COVID-19 pandemic expedited the process since with lockdowns it became impossible to implement the program in person. During the testing phase of the conversion of the model to being fully virtual, ARC found that they had more participation from fathers, who could participate in sessions with their wives on Sunday nights and have mentoring sessions as late as 10 PM. The ARC staff believed that it was important to be flexible in its approach so they could support more families.

The virtual HEPPP content currently includes 38 sessions covering a wide range of content:

- 20 sessions on parenting cover: introduction to ECD; physical, cognitive, social, and emotional development; brain development; communication between caregivers and children; communication with siblings and peers; behavioral problems in early childhood,; self-control; use of a non-violent approaches with children; reinforcement of positive behaviors; play; safety and prevention of accidents; personal hygiene and protection from diseases; balanced nutrition; reinforcing justice and equality between children; multiple intelligences and learning; kindergarten and school preparedness; sexual education; families of children with special needs; and life skills.
- Six sessions on early learning cover: introduction to early learning, challenges of early learning in the home; mental health of children and caregivers; learning languages at home; learning science and math at home; arts and culture.
- Six sessions on mental health for caregivers cover: introduction to mental health; stress from a caregiver's point of view; relationship with self; family dynamics; couple relationships; positive communication within the couple; expressing emotions; gender equality; self-awareness; emotional regulation.
- Six sessions on mental health of children for parents and caregivers are under development and will focus on the importance of caring for the child's mental health, personality development in the early years, building a growth mindset, and building resilience.

The virtual sessions include a variety of types of content relevant to Middle Eastern cultures. These include videos of interviews with experts and parents; educational, informative, and scientific videos in simple and easy-to-understand language; animated videos; graphics and infographics; interactive games and activities that build knowledge and skills; audio clips; and multiple-choice questions.

Parents are required to participate independently in 2 sessions per week, each of which lasts between 20–40 minutes. Individual mentoring calls for couples, lasting an average of 20–30 minutes, accompanies each session. The full program lasts about two and a half to three months. While all 32 sessions are available for all parents to learn from independently, ARC now provides mentoring calls for 20 sessions as a mandatory base to achieve changes in attitudes, norms, and behavior changes. Ten sessions laying the foundation on the importance of ECD and parental engagement are mandatory, but each new cohort of parents can select 20 additional topics they want to focus on based on their most pressing parenting needs. Anyone who wants to learn more can do the additional sessions independently, or seek more information on ARC's online parenting hub, Urjouha[1] (*Swing* in Arabic), an open-sourced website that has resources to support families and caregivers of children up to eight years. There are also articles, infographics and tips on social media platforms (Instagram and Facebook) and videos on YouTube.

To deepen the learning, ARC includes individual mentoring and peer-to-peer support through WhatsApp groups. Mentoring is a key element of the virtual delivery of HEPPP as it helps reinforce the online sessions. ARC has been testing different types of mentoring – with master trainers and also trained parent graduates. WhatsApp groups for the parents enrolled in the online sessions allow them to connect with their mentors and other parents as well. In 2021 the full HEPPP model became available online and available for anyone to access. At the end of the program, participants can obtain a certificate of completion.

HEPPP program evaluation research design

Between 2019 and 2021, ARC conducted research on three of its implementation modalities – two face-to-face versions and a fully virtual one – to inform implementation and potential future iterations of the model. This section describes the intended and final research design.

ARC implemented HEPPP in Lebanon through local Community-Based Organizations (CBOs) and municipalities and worked with Plan International as a sub-grantee to implement HEPPP in Jordan. American University in Beirut provided technical assistance for the research design and Modern University collected data for the study. In Lebanon, implementation and research were in the Bayssour area in Mount Lebanon and Bourj Al Barajneh camp in Beirut. In Jordan, implementation and research were in the Azraq refugee camp. Most of the participants in the implementation and research were Syrian and Palestinian refugees or vulnerable Lebanese and Jordanian families.

ARC and the American University of Beirut initially designed the research as a randomized controlled trial (RCT) with three groups – one control and two implementation groups. The aim of the RCT was to compare different delivery modalities with underprivileged host communities and Syrian and Palestinian refugees in Lebanon and Jordan. The program was intended to directly improve the knowledge, behaviors, and practices of parents and improve children's developmental outcomes and well-being. The evaluation looked at couples (usually mothers and fathers) and different aspects of their parenting, such as the way they disciplined their children and managed their children's positive and negative behaviors.

The implementation and research, which were conducted simultaneously, started in 2019. At baseline, the research team collected in-person pre-program data with two face-to-face cohorts. ARC had already planned on digitizing the HEPPP and the COVID-19 crisis accelerated this process. So, a few months after the face-to-face cohorts were started, ARC began implementing an online version as well. While not originally in the initial research design, ARC decided that it was worth including the online cohort as part of the RCT. Baseline data for the online version was also collected for the research, but as this happened early in the COVID-19 pandemic, before lockdowns happened, they collected this data in-person using physical distancing and masking.

Unfortunately, the team was unable to collect sufficient pre-intervention data on the control group. ARC did not have funding to do a wait-list control and so could not offer the services after the completion of the research to the control group. The control group participants realized that other families were receiving services and did not want to give their time for data collection when they were not receiving services. Without a control group, it was not possible to conduct an RCT, so ARC had to re-design the research.

With the onset of COVID-19 lockdowns in February 2020 in Lebanon and Jordan, parenting programs shut down. This caused additional challenges with the post-intervention data collection. With lockdowns, the research team could only collect post-intervention data virtually, which many participants did not want to do. Parents, particularly male participants, said they did not have the time to respond to surveys and felt that some of the questions were too personal. This all led to attrition among all groups.

This experience highlights important lessons learned for conducting research in crisis situations and where an agency wants to collect data with a control group, but cannot provide services to them at a later date. Despite attrition among all cohorts, the study was still to collect sufficient post-intervention data to do some analysis and share results.

Updated study design and participants

The final study design used mixed methods, with pre- and post-intervention data from participants in three program cohorts – two face-to-face and one online. The two face-to-face interventions had similar content and were conducted in the same time period. However, facilitators were different for the two groups. Group 1 was delivered by parents who participated in the HEPPP as parents and were trained to become community facilitators, called *Sanads* ("support" in Arabic). Group 2 was delivered by master trainers (both male and female) with a background in psychology and ECD. Both groups involved approaches that built on parents' current practices and knowledge through interactive activities like brainstorming activities, group work, role playing, case studies, short presentations, and focus group discussions. Group 3 was an online self-paced intervention facilitated by a combination of both parent-trained and master-trainer mentors. The difference between the master trainers and trained parents was that the mentors had more experience in supporting parenting programs and had deeper ECD expertise. The sessions were held once weekly (on Friday night, or Saturday and Sunday during the day) for the face-to-face modality (i.e., for four to five months), and twice weekly for the online modality (i.e., over two months).

The target population for the three modalities and the study included refugees from Syria or Palestine and underprivileged Lebanese and Jordanian families who satisfied the following criteria: (1) they have children aged eight years or younger: (2) they were parents in couples unless unfeasible (e.g., traveling partner, widow, missing partner, divorced); (3) they understood the Arabic language and; (4) they could participate in the complete set of weekly sessions and discussions, each lasting 90 to 120 minutes. Parents of children with self-reported psychotic disorder, disruptive behavior disorder, or substance abuse were excluded from the study.

The directors of community and social service centers (identified as community leaders) located within each camp and host community facilitated the recruitment process under the supervision of four ARC and Plan International research coordinators. The research coordinators invited eligible parents to participate in these interventions after explaining the objectives and phases. Furthermore, they shared benefits and risks (e.g., changing the schedule of sessions that may delay the end of the intervention). They also explained that participation would be voluntary and that all information exchanged during face-to-face or online interviews would be strictly confidential. Parents who agreed to participate in the face-to-face interventions (Groups 1 and 2) signed informed consent forms before the first interview with the data collectors. The parents participating in the online intervention (Group 3) gave oral permission to a similar informed consent text. The recruitment of more participants for face-to-face Groups 1 and 2 stopped because of the COVID-19 pandemic. Participants were given transportation fees after every face-to-face session to support their participation. The recruitment stopped in the online intervention once the required sample size was reached. Parents who accepted participation in the online intervention attended self-paced online sessions and then answered subsequent phone calls with mentors (master trainers and trained parents).

A total of 58 parents were recruited for Group 1, including 24 couples; 89 parents in Group 2, including 42 couples; and 235 parents in Group 3, including 116 couples. Most of the recruited parents in Groups 1 and 2 attended at least 70% of the face-to-face sessions, while participants in Group 3 attended all the online sessions. A three-month post-intervention follow-up was conducted on 98% of the parents (including 22 couples) from Group 1, 94.4% of the parents in Group 2 (including 39 couples), and 86% of the parents in Group 3 (including 96 couples). Non-couple participants were mainly mothers whose husbands were not in the same country during the intervention period. Study attrition was mainly due to a change in life circumstances (e.g., finding a job) or living place, mainly after the COVID-19 pandemic and the Lebanese crises that started at the end of 2019. Table 5.1 illustrates the three cohort groups for the research.

Table 5.1 Research cohorts

Group	*Facilitator*	*Participants*	*Types of sessions*
Group 1 (face-to-face)	Led by parent-trained mentors who graduated from the program before (peer-to-peer approach)	58 parents (including 24 couples plus single parents)	16 responsive parenting sessions
Group 2 (face-to-face)	Led by master-trainer mentors with support from other experts	89 parents (including 42 couples plus single parents)	16 responsive parenting sessions plus 5 psychosocial support sessions
Group 3 (online)	Self-directed with support from parent-trained and master-trainers mentors	235 parents (including 116 couples)	16 responsive parenting sessions

Study procedures and instruments

The study used a mixed methods approach. There were 7 quantitative tools (see Table 5.2). Qualitative tools included focus group discussions (FGDs) and key informant interviews (KIIs).

These quantitative tools were selected in part because they were already available in Arabic and contextualized and validated for the context. Furthermore, ARC had used these tools before during a previous evaluation conducted with Yale University.

Survey questionnaires were also used by data collectors on the team. The structured interview was designed based on an earlier research project entitled "Mother-Child Education Program" conducted in partnership with Yale University in the United States (Ponguta et al., 2020). It included parents' demographics, including gender and age, and their child's gender and age. Because some of the participants were illiterate, the interviewer read aloud each question from the printed questionnaire during each interview, which lasted around 40 minutes. When a participant exhibited uncertainty about a particular issue, the interviewer simply offered brief explanations, usually by paraphrasing the question. The mother and father were interviewed separately in different rooms and/ or at different times so they would not hear each other's answers. Participants were reminded during the interviews that their answers would be kept confidential, that there were no right or wrong answers to the questions, and that their responses would have no bearing on their participation or any benefits they received or may receive from the community and/ or social service centers, and that it was vital for them to answer as honestly as possible.

Qualitative data collection was limited by restrictions on meeting face-to-face during the post-intervention data collection period. Data collectors therefore had to use phone calls which required converting focus group discussions to key informant interviews. With multi-family households being the norm, some families may have been in the same room as extended family members, children, and others, during the calls so the interviews were not always private so answers could have been biased. The research team found it challenging to obtain qualitative data that covered participants' holistic experience.

Results and analysis

Results

At baseline, there were more mothers than fathers in each group and more boys than girls. The mean age of parents was 36.3 years in Group 1, 35.5 years in Group 2, and 35.3 years in Group 3 (see Table 5.3) The mean age of children was 3.4 years in Group 1, 3.9 years in Group 2, and 4.0 years in Group 3 (see Table 5.3).

Table 5.2 Quantitative tools

Instrument Name	*Description*
The WHO (five) Well-being Index 1998 version (WHO-5)	• A short five-item self-administered screening instrument that measures psychological well-being including depression among adults in 2–3 minutes. • Used extensively worldwide, translated and validated in different languages, including Arabic. • Each of the five positively worded items is rated on a 6-point Likert scale ranging from 0 (constantly present or all of the time) to 5 (not present or at no time). The summed scores range from 0 to 25. • A score < 13 indicates poor wellbeing or low mood, though not necessarily depression and warrants further assessment.
Patient Health Questionnaire- 2 items (PHQ-2)	• Assessment of degree to which an individual has experienced depressed mood and anhedonia (loss of pleasure) over the past two weeks, on a 5-point Likert scale from 0 (not at all) to 3 (nearly every day). • The higher the score ≥ 3, the more likely there is an underlying depressive disorder
MOS Social Support Survey (MOS-SSS)	• A 19-item multidimensional, self-administered instrument developed for patients in the Medical Outcomes Study (MOS). • Measures the availability of support, if needed, in several domains, on a 5-point Likert scale from 0 (I never find it) to 4 (I find it all the time). • The 19-items comprise four subscales (emotional/informational support: eight items, tangible/material support: four items, affectionate support: three items, positive social support: three items) and overall summary index (overall social support). One item (item 13) is not included in any subscale, but is included in the summary index
Family Involvement Questionnaire (FIQ) Home-Based Involvement Dimension	• Assesses the partner's behaviors with the participant and with his/her children. It includes behaviors describing the active promotion of a learning environment at home for children, such as providing a place in the home for learning materials and creating learning experiences for children in the community. • Participants are asked to rate how frequently they perform each of the 13 behaviors on a 5-point Likert scale from 1 (strongly disagree) to 5 (strongly agree). • The parent's total score is calculated. The parent's positive behaviors with partner are summed. The parent's negative disciplines with children are summed.
The Parenting Stress Index-Short Form (PSI-SF)	• A 36-item self-report questionnaire of parenting stress developed from the full PSI 120-item (duration: 10–15 minutes). • One of the most commonly used measures of parenting stress among parents of children younger than 12 years. It is validated in Arabic language. • Items are rated on a 5-point Likert scale from 1 (strongly disagree) to 5 (strongly agree). • Includes four subscales: Defensive Responding (DR), the Parental Distress (PD), Parent-Child Dysfunctional Interaction (PCDI) and Difficult Child (DC) subscales, and a Total Stress scale with internal consistency reliability coefficients of 0.90, 0.89, 0.88, and 0.95 respectively. • Higher scores indicate greater levels of parenting stress.

Instrument Name	*Description*
The Strengths and Difficulties Questionnaire (SDQ)	• A widely used brief screening questionnaire for psychosocial problems and strengths in the child's daily life. • Appropriate to use in children between four and 16 years. • The Arabic version of the Children SDQ, was validated with parents and teachers of Yemeni children. • SDQ asks about 25 attributes with three response categories (0 = not true, 1 = somewhat true and 2 = certainly true), some positive and others negative to cover a broad range of mental health symptoms. It includes five impact scales: conduct problems (fivr items), hyperactivity-inattention (five items), emotional symptoms (5 items), and peer relationship problems (five items), as well as prosocial behaviors (fivr items). • The first 20 items (i.e., without the prosocial behaviors items) are summed to create a "total difficulty" scale score ranging from 0 to 40, with "high" SDQ scores predicting much greater rates of mental disorders than "low" scores.
Child Disciplines - UNICEF MICS6	• Child discipline module contains questions about 11 child discipline behaviors • The mother or caretaker of a child under five is asked if s/he or another adult in the household has used the discipline method within the past month. Answers can only be yes or no.
Parental Discipline Strategies Questionnaire (DSQ)	• Assesses 18 discipline strategies grouped into seven discipline dimensions: inductive discipline, physical punishment, manipulation of privileges, harsh verbal discipline, argument, shaming, and ignoring. • Parents are asked how frequently they use each of the 18 different discipline strategies in the last year on a 5-point scale (1 = never, 2 = less than once a month, 3 = about once a month, 4 = about once a week, 5 = almost every day). • The parent's total score of each discipline dimension is calculated.

Sources: (Abidin, 2012; Alyahri and Goodman, 2006; Croft et al., 2015; Dafaalla et al., 2016; Gilbody et al., 2007; Goodman, 1997; Goodman, 1999; Goodman, 2001; Goodman and Goodman, 2009; Huang et al., 2011; Marquis and Flynn, 2009; Sherbourne and Stewart, 1991; Sibai et al., 2009; Stone et al., 2010; Topp et al., 2015; WHO, 1998).

Table 5.3 Mean age of participants

	Mean Age of Parents (in years)	*Mean Age of Children (in years)*
Group 1	36.3	3.4
Group 2	35.5	3.9
Group 3	35.3	4.0

Group 1 results

For Group 1 (face-to-face intervention by parent-trained mentors), the post-intervention follow-up assessment at 3- months found significant improvement in several scales and subscales. According to two measures, parental distress was lower (i.e., PSI-SF (p: 0.003, r: 0.29), and SDQ total difficulties (p: 0.005, d: 0.76)). SDQ per item analyzed revealed a significant decrease in a few children's problematic behaviors, including one emotional behavior (i.e., "the child has many fears, is easily scared") and two conduct behaviors (i.e., "the child often lies or cheats in children > 4 years," or "often argues with adults in children between 2 to 4 years," and "the child steals from home school or elsewhere in children > 4 years," or "can be stingy with others for those in children between 2 to 4 years.") DSQ per item analyzed revealed a significant decrease in parents practising all physical punishment disciplines (i.e., "spank, slap or hit your child, grab or shake your child," and "throw something at your child"), one harsh verbal discipline (i.e., "try to scare your child into behaving well, e.g., threaten to call the police or tell the teacher?"), and one disciplinary argument (i.e., "threaten your child with some punishment?"). The UNICEF MICS6 parental disciplines per item analyzed revealed a significant increase in practicing one positive discipline (i.e. "took away privileges, forbade something he/she liked, or did not allow him/ her to leave the house") and a significant decrease in practicing two harsh verbal disciplines (i.e., "shouted yelled at or screamed at him/ her," and "called him her dumb lazy or another name like that,") and one physical punishment discipline ("hit or slapped him/ her on the hand, arm or leg.")

Group 2 results

For Group 2 (face-to-face intervention by master-trainer mentors), the post-intervention follow-up assessment at 3- months found a significant improvement in the following scales and subscales: WHO-5 total score (p: 0.034, d: 0.19), PSI-SF total score ($p < 0.001$, d: 0.65). There were also improvements in several dimensions of the PSI-SF, including reduced defensive responding ($p < 0.001$, d: 0.40), parental distress ($p < 0.001$, d: 0.45), parent-child dysfunctional interaction (p: 0.001 d: 0.35), and difficult child behaviors ($p < 0.001$, d: 0.50). The SDQ per item analyzed revealed a significant improvement in one emotional behavior (i.e., "the child has many worries or often seems worried"). The DSQ per item analyzed revealed a significant increase in one disciplinary argument (i.e., "argue or quarrel with your child"), one shaming discipline (i.e., "say you are disappointed with your child or say that his or her misbehavior hurt your feelings"), one manipulating discipline (i.e., "give your child extra chores"), and a significant decrease in practising another manipulating privilege (i.e., "promise a treat or privilege to your child for good behavior") and two physical punishment disciplines (i.e., "spank slap or hit your child" and "throw something at your child"). The UNICEF MICS6 parental disciplines per item analyzed revealed a significant decrease in practising one physical punishment discipline (i.e., "hit or slapped him/ her on the hand, arm or leg").

Group 3 results

For Group 3, (online intervention by a combination of both parent-trained and master-trainer mentors), the post-intervention follow-up assessment at three months found a significant deterioration in the MOS-SSS tangible/material support, which includes things like a spouse helping with basic needs if one was incapacitated (p: 0.032, d: 0.15). Meanwhile, there was a significant improvement in the following scales and subscales: WHO-5 ($p < 0.001$, d: 0.36), PHQ-2 ($p < 0.001$, d: 0.35), PSI-SF total score ($p < 0.001$, d: 0.31), PSI-SF defensive responding ($p < 0.001$, d: 0.40), PSI-SF18 parental distress ($p < 0.001$, d: 0.46), and PSI-SF difficult child behaviors (p: 0.025, d: 0.19). However, there was a deterioration in the SDQ prosocial scale, which measures children's resources ($p < 0.001$, d: 0.46). The SDQ per item analyzed revealed a significant deterioration in different prosocial strengths, and a significant improvement in one conduct behavior (i.e., "often fights with other children or bullies them"), one peer behavior (i.e., "general liked by other children"), and two emotional behaviors (i.e., "nervous or clingy in new situations easily loses confidence" and "has many fears easily scared"). The DSQ per item analyzed revealed a significant decrease in practicing one parental manipulating privilege (i.e., "promise a treat or privilege to your child for good behavior") and one disciplinary argument (i.e., "raise your voice yell at or scold your child"). The UNICEF MICS6 parental disciplines per item analyzed revealed a significant decrease in explaining to the child why a behavior was wrong and a significant improvement in different parenting negative disciplines; however, the number of parents who answered "don't know" or "no" to the question "Do you believe that in order to bring up raise or educate a child properly the child needs to be physically punished?" increased significantly.

Box 5.1 Success story: Violence reduction in home

Ahmad and Rawiya are parents of a family living in Borj El Barajneh camp, Lebanon. Both had trouble at the start of the online modality keeping up with the sessions. The trained parent facilitators following up with them started having phone calls with them to help them go over the session activitiss. The couple progressively started to share more, and Ahmad shared that he hit his children. After session 7 "Avoiding violence & harm," Ahmad gathered his entire family and made a promise to them and to the mentor that he would change his behavior. At the end of the project, the children joined the call again and thanked the mentor, telling her about how Ahmad and Rawiya's behavior had changed, and how Ahmad stopped using violence at home.

Box 5.2 Success story: Father expressing emotions

Maher and Amani are parents of five children living in Jordan. After participating in the project, Maher reached out to the team to thank them for helping him overcome the taboos and pressure from society. Maher said "For men, it is hard to express emotions, it is usually interpreted as weakness. This is the first time I have done something like this and it made me feel much lighter and more engaged with my family. They understand me better and I do too. My children became more affectionate towards me and I spend a great time with them"

The results of the evaluation revealed some positive changes in parenting knowledge, behaviors and practices, with variation in both virtual and face-to-face modalities. Because there was no control group it is not possible to identify the impact of any of the interventions alone.

Even though it was implemented during conditions of uncertainty and overall stress for parents – the COVID-19 pandemic and the subsequent crises in Lebanon – the online intervention conducted by a combination of both parent-trained and master-trainer mentors had the highest participation and completion rates. This could be because parents could work on the course when they had time and make their own schedules. This model was the most effective in improving parents' well-being (i.e., improvement in their mood). It also significantly reduced their parenting stress and their negative discipline practices, meaning fewer used physical punishment. Nevertheless, based on parental feedback, many of their children's social skills declined after the online intervention, possibly due to the prolonged lockdown during the early period of the COVID-19 pandemic even though their parents were more engaged with their children.

The face-to-face versions, supported by the same master-trainer mentors as the online version, had an even more positive effect on reducing stress and improving parents' psychosocial well-being. The face-to-face versions also allowed parents to practice ways they could interact and play with their children with the support of mentors and other parents.

The face-to-face intervention led by master-trainer mentors had a dropout rate of 25%, but still had positive effects on parents' psychosocial well-being and stress levels which improved more than in the online program. This could be because the face-to-face approach allowed parents to get more peer to peer support than online participants who got this through WhatsApp.

Challenges in implementing the face-to-face modality mainly included outreach and dropout for fathers. This challenge was resolved in the online methodology, as it opened the platform for fathers to participate by providing flexible opportunities to participate that would not impact their work schedules and removing the stigma of going to a community-based center.

Reflections for the future

There are many lessons for ARC to consider for the future iteration and implementation of HEPPP. Through the process mentioned in this chapter, the team identified five areas of reflection and modification. These are related to the actual evaluation tools used, the content of HEPPP, the delivery modality, the inclusion of mentors and Sanad parents, and the target audience.

While all of the tools used in this evaluation were used previously during a partnership and research conducted with Yale University, the team realized a few things that they will modify for the future. The team used a lot of tools and this sometimes required a lot of parents' time, which affected their engagement and response bias. ARC and its partners also realized that providing multiple options (i.e., scales of 1 to 5 on tools that ranged from agree to don't agree) were too onerous and perhaps scales with fewer options (i.e., 1 to 3) would be better. The data collection, at times, felt rushed and it seemed that parents just wanted to quickly answer and be finished. This could have affected the answers the parents gave. In the future, ARC will not spend more than 20–25 minutes collecting data in order to retain parents' attention and will reduce the options on tools to options 3. By reducing the time for collecting data from each parent/couple, parents may provide more thoughtful answers.

ARC also learned that some of the wording of the various tools was too complicated for parents to understand, even in already validated and contextualized tools. Therefore, there is a need to do more testing of the tools. Some questions also made parents feel uncomfortable. For example, questions asking couples if they hugged or showed affection should be revised or removed they made some couples feel uncomfortable.

After starting the implementation and the research of the three above-mentioned modalities of HEPPP, ARC decided to develop a hybrid version once the COVID-19 lockdowns eased. The hybrid version could retrain many online modules, such as content areas about child development, but also support parents' own stress reduction and psychosocial well-being in-person. ARC believes that a hybrid approach might be ideal for ensuring positive changes among parents for knowledge, behaviors, and practices about child development and positive psychosocial well-being. This hybrid version will be tested in the future (2023).

Another reflection and learning after the evaluation of the face-to-face and virtual modalities was what and how the content can continue to be further refined. ARC has decided that the next version of the HEPPP curriculum, either implemented face-to-face, virtually, or through a hybrid approach, will include elements of children's mental health, children's disabilities, and more emphasis on learning through play. ARC found that while HEPPP has a focus on psychosocial well-being, it is only for the parents and not the child. So, these elements need to be added and are currently being developed. ARC also reflected on strengthening and deepening the psychosocial sessions for parents and developing them into deeper Caring for the Caregiver sessions. At the same time, ARC will continue to test these content areas to understand if they do indeed make a real difference and are worth the cost. ARC may decide that some new content areas should be core modules or that they should be optional ones.

Each of the delivery modalities had some sort of mentorship support, but there was a difference in the skills and experience of the mentors and the dosage of mentorship provided. Group 1 had parent-trained mentors whereas Group 2 had master-trainer mentors who were either ARC staff or those with deeper knowledge of ECD and more years of experience supporting parenting programs. Group 3 had a combination of both parent-trained and master-trainer mentors with greater experience and expertise. While the results from the evaluation found that the master-trainer led delivery had better impacts on parents' knowledge, behaviors, and practice than parent-led ones, ARC still needs to understand how much better. To date, ARC has not done an RCT or a cost-effectiveness analysis. A cost-effectiveness analysis could help ARC determine whether parent-led mentors are best based on both cost and the outcomes. These research approaches might be important next steps as ARC determines the best combination of type of mentor, content, duration etc.

Lastly, an unintended consequence of ARC's implementation of the HEPPP model is that professionals like ECD and early-grade teachers also began taking online courses through the Urjouha website. One teacher told ARC that the course shifted her approach with her own children at home and it helped her improve her teaching with her young students. A school principal in Lebanon has asked ARC if they could train the teachers in her school. To respond to requests such as these, ARC has now started working with ECD professionals using the HEPPP package.

Conclusion

Over the past decade, the Arab Resource Collective (ARC) Parenting Program "HEPPP" has been under continuous development, review, monitoring, and improvement. To support parents experiencing multidimensional crises in Lebanon and Jordan over the years, ARC developed three delivery modalities: two face-to-face versions (one delivered by parents and another delivered by master trainers), and one virtual. Research that ARC and its partners conducted after the COVID-19 pandemic between 2019–2021 on refugees from Syria and Palestine and host communities from Lebanon and Jordan highlighted outcomes from each of the modalities.

ARC learned the myriad challenges of using rigorous research designs in multi-crisis contexts where people are moving and where a waitlist control group is not possible. The randomized controlled trial ARC first envisioned was not possible to implement, but a mixed methods pre-post research design on participating families was possible. ARC learned a great deal about existing data collection tools that were already contextualized to the Middle East context and made notes of further modifications required for the future. Future research should still attempt an RCT or similarly rigorous research design along with a cost-effectiveness analysis.

The content, duration, and delivery modality also provided significant learning. At times, the team had to be flexible and quickly change direction. In future iterations of the development and refinement of HEPPP, there are many content areas ARC can add, reduce, or modify. While all three models improved parents' knowledge, behaviors, and practices, the online version enabled greater participation, especially of fathers, while the face-to-face version offered greater support for parents to reduce their own stress and improve their own psychosocial well-being.

While targeting parents and caregivers, ARC also found that other adults, such as ECD and early-grade teachers, tried the online HEPPP model and requested more support so the model could also be tested to strengthen the ECD and early-grade teaching workforce. So, HEPPP could be used in the future for strengthening the ECD workforce in the future. ARC has already begun testing this.

In sum, the learnings from this chapter are not only relevant for Lebanon and Jordan but are relevant to other countries in the Middle East and around the world. While HEPPP has been developed and implemented mostly in multi-crisis contexts, most of the content can be used for non-crisis or lower-risk situations as well as much of the core content in non-emergencies and emergencies would be the same. The learnings of what worked and did not work with the different modalities are also something other implementing agencies can learn from as they develop their own parenting programs. Academics and researchers can learn about the challenges of doing research in unstable multi-crisis contexts while policymakers and donors can use the initial evidence provided in this chapter as they make future policy and funding decisions.

Note

1 "Urjouha" is an open source, comprehensive online free platform for parenting babies, toddlers, and pre-schoolers in Arabic with different parenting materials, including videos, articles, infographics, podcasts and more. www.urjouha.net.

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